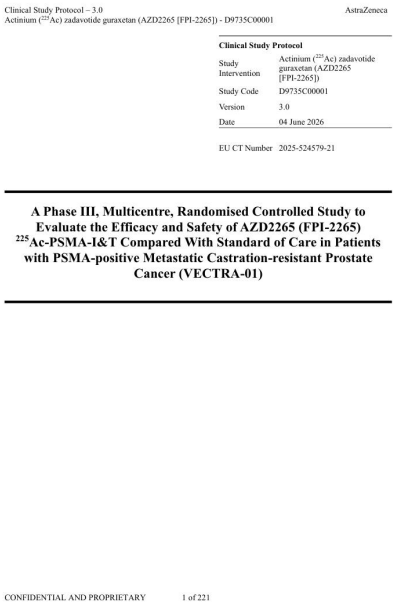
NucMed Board Studio · protocol study sheet

VECTRA-01

D9735C00001 · ^{225}Ac-PSMA-I&T vs SoC · Phase III

AstraZeneca · v3.0 04 June 2026 · 221 pages



Clinical Study Protocol – 3.0 AstraZeneca
Actinium (^{225}Ac) zadavotide guraxetan (AZD2265 [FPI-2265]) - D9735C00001

Clinical Study Protocol

Study Intervention	Actinium (^{225}Ac) zadavotide guraxetan (AZD2265 [FPI-2265])
Study Code	D9735C00001
Version	3.0
Date	04 June 2026

EU CT Number 2025-524579-21

A Phase III, Multicentre, Randomised Controlled Study to Evaluate the Efficacy and Safety of AZD2265 (FPI-2265) ^{225}Ac-PSMA-I&T Compared With Standard of Care in Patients with PSMA-positive Metastatic Castration-resistant Prostate Cancer (VECTRA-01)

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What this study is, in one breath

Randomise PSMA-positive mCRPC (after ARPI, taxane, and one PSMA beta) 1:1 to 12 MBq actinium every 6 weeks or investigator-choice SoC. Dual primary rPFS and OS.

Alpha therapy. Not Pluvicto. Not the diagnostic PSMA PET used to enter the trial.

> **If the protocol feels like a wall of words**
>
> A protocol is a recipe: who can join, what is injected, how much, when you scan or treat, and how you decide if a picture or outcome was right. This sheet is the same idea in normal English. It does not replace the signed site copy.

1. What VECTRA-01 is

AstraZeneca D9735C00001: open-label Phase III of ^{225}Ac zadavotide guraxetan (AZD2265 / FPI-2265, PSMA-I&T alpha) versus investigator-choice standard of care in PSMA-positive mCRPC after ARPI, a taxane, and one PSMA beta radioligand. Coordinating investigator Morris. ~670 randomised 1:1, ~1,120 screened. No crossover.

Arm	What they get
A	12 MBq IV ^{225}Ac-PSMA-I&T every 6 weeks, up to 6 cycles (≤ 36 weeks). Fixed activity, not weight-based.
B SoC	Investigator choice: cabazitaxel 20 mg/m² Q3W (+ pred 10 mg), or ARPI (enza 160 / abi 1 g), or radium-223 55 kBq/kg Q4W × 6.

Stratify on intended SoC, number of taxane courses (1 vs >1), and timing of beta-PSMA progression. IDMC watches safety, efficacy, futility. Dual primary: rPFS and OS.

2. PET gate (not the IMP)

- Central BICR must call the patient PSMA-positive before randomisation.
- ^{68}Ga-PSMA-11 111–259 MBq (preferred; EU) or ^{18}F-DCFPyL 296–370 MBq (not in EU). Same tracer at screen and follow-up. Window 28 days.
- Those diagnostic tracers are auxiliary products. The IMP is 12 MBq actinium.
- Prior PSMA beta RC allowed if ≥ 3 months from last dose. No prior alpha-targeted RC.

3. Renal and salivary gates (v3.0)

- eGFR ≥ 60 mL/min (tightened from 50 after EMA). AKI, CKD, uncontrolled DM excluded.
- Xerostomia Grade 1 allowed; ≥ Grade 2 excluded (salivary is an OAR for PSMA alpha).
- Marrow: Hb ≥ 9, platelets ≥ 100, ANC ≥ 1.5, ECOG 0–2.

Not Pluvicto, not radium-223

Same PSMA motif as ^{177}Lu-PSMA-I&T, but alpha instead of beta. Kidney and salivary glands take a harder hit than Lu-177.

What to know cold

Q. IMP?
A. 12 MBq ^{225}Ac zadavotide guraxetan IV Q6W, up to 6 cycles.

Q. Control?
A. Cabazitaxel, ARPI, or radium-223. No crossover.

Q. PET gate?
A. BICR PSMA-positive on Ga-11 or DCFPyL. Those are not the IMP.

Q. Renal?
A. eGFR ≥ 60. Xerostomia ≥ G2 excluded.

Q. Primaries?
A. rPFS and OS. ~670 randomised.

How to use this with Board Studio

Open the matching protocol module and press play. The left pane is the signed page. Use this PDF when legalese stalls you. If anything conflicts, the signed protocol wins.

Disclaimer. Educational summary for board-style study. Not medical advice, not a recruitment brochure, not a substitute for the signed protocol, and not affiliated with the sponsor, ABR, ABNM, ACGME, SNMMI, NRC, or FDA. Confirm numbers against the current site copy before any clinical use.